NURSING PROGRESS NOTE

Respiratory Imaging Follow-up · SOAP Format

Case ID	RNP-7A9C	**Setting**	Med-surg observation
Patient	55-year-old female	**Documentation date**	June 9, 2026
Source reviewed	Chest radiographs incl. right-side-down decubitus view	**Note status**	Pre-treatment-decision

Documentation boundary: This note records imaging-supported findings and patient-reported symptoms only. The finalized written radiology report is the authoritative source and takes precedence over image appearance. No vital signs, oxygen saturation, laboratory results, medications, allergies, comorbidities, or treatment orders were included in the materials provided; none are inferred. This nursing note does not establish a medical diagnosis, severity grade, or treatment plan.

S – SUBJECTIVE

55-year-old female on observation status reports **progressive shortness of breath and cough**. No additional subjective detail (onset timing, duration, severity rating, aggravating or relieving factors, or associated symptoms) is documented in the available materials.

O – OBJECTIVE

Imaging reviewed: chest radiographs with right-side-down decubitus view (Case RNP-7A9C). The finalized written radiology report is treated as authoritative.

Imaging-supported findings (per finalized radiology report):

- Moderate-sized **right pleural effusion** that freely layers on the right-side-down decubitus view (free-flowing fluid).
- Hazy opacification at the **right lung base**; per the report, favored to represent a combination of layering pleural fluid and atelectasis.
- No pneumothorax.
- Mild enlargement of the cardiopericardial silhouette.
- Median sternotomy changes (imaging evidence of prior sternotomy).
- Radiology impression: **simple pleural effusion**.

Data not available in provided materials:

- Vital signs, oxygen saturation, laboratory results, medications, allergies, and comorbid history were not included.
- No independent nursing physical-assessment findings (respiratory rate, lung auscultation, work of breathing, SpO_2) are recorded in the materials available for this note.

A – ASSESSMENT (nursing)

Nursing assessment is limited to the reviewed imaging and patient-reported symptoms and does not constitute a medical diagnosis.

Imaging-supported concern:

- A moderate-sized right pleural effusion is present on imaging. Pleural fluid can limit full lung expansion and is a plausible contributor to the reported shortness of breath and cough. This symptom-to-imaging linkage is stated cautiously and is not a measure of physiologic severity.
- The radiology impression of a simple, free-layering effusion describes the fluid's behavior on imaging, not its cause.

Clinical uncertainties requiring provider follow-up (not established by imaging alone):

- The cause (etiology) of the pleural effusion is not determined by imaging.
- The right basilar opacity is reported as ‘favored’ to represent layering fluid plus atelectasis – an imaging interpretation rather than a confirmed process; other contributors are not excluded by imaging.
- The clinical significance of the mild cardiopericardial silhouette enlargement requires provider correlation.
- Physiologic severity, oxygenation, and respiratory status cannot be assessed without vital signs, SpO_2, and laboratory data, which are not available.
- No provider diagnosis or treatment decision is documented at this time.

P – PLAN (nursing actions; not medical orders)

- Communicate the reviewed imaging findings to the provider; request provider evaluation for effusion etiology and a management decision (none documented to date).
- Provider to correlate imaging with clinical examination, vital signs, oxygen saturation, and laboratory data (not included in current materials).
- Maintain nursing monitoring of respiratory status appropriate to observation status; promptly escalate to the provider any worsening shortness of breath, increased work of breathing, chest pain, decreased oxygen saturation (if measured), or change in level of consciousness.
- Provided patient and family education on pleural effusion and its possible relationship to breathing symptoms, including when to seek urgent help (see patient education summary); reinforced that next steps require provider follow-up.
- No oxygen, medication, or procedure is ordered or implied by this note.

RN signature: _____ Credentials: _____ Date / Time:

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Prepared by nursing for the medical record.